



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : فله محمود سيد محمود
المحافظة : القاهرة
الحى / المدينة : رمسيس
رقم البطاقة : 28403262101382 رقم التليفون : 01019079266
العمر (تاريخ الميلاد) : 26-03-1984 المهنة : ربه منزل
العنوان : 20
تاريخ تسجيل الإستمارة : 29-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي خط علاج كيميائي ما بعد الجراحه طبقا
لبروتوكول (Gemcitabine+Platinum) [14254]
قسم العلاج :
الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: فله محمود سيد محمود
National ID: 28403262101382



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Adjuvant gemcitabine plus platinum chemotherapy

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

فله
محمود سيد محمود عيسى
ميت رهينه
مركز البدرشين - الجيزة

١٩٨٤/٠١/٢٦ ٢٨٤٠٣٢٦٢١٠١٣٨٢

IG3384103

١٢٣٤
٥٦٧٨

٢٠١٩/١١ ٢٨٤٠٣٢٦٢١٠١٣٨٢

ربة منزل

أنش
مسلمة
مكتوبة
صلاح فهمي محمد احمد حسن

البطاقة سارية حتى ٢٠٢٢/١١/١٩

١

05/24/2026

6

MDT FORM

Name:	Euahh Mohamed El-Sayed Mohamed	MRN:	4581
Age:	42 Y	ID:	28403262101392
Menopausal Sta		Phone:	01019079266
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: IDC
ER: 8/8 PR: 8/8 HER2: 0 KI67:

R

L

Sonomammography:

Mets Workup:

History:

MBC to bone and liver since 2022, Rec Taxol-Carbo 7 cycles 2022 RD
Femara + Zoladex till 2024 DP : Faslodex+Ibrance 8-->11 2024 DP.
Xeloda 3 cycles DP .. Taxotere 12 cycles then 1-2026 DP Rt Axillary LN biopsied
acc to MDT on 2-2026: same behavior breast origin ER 8/8, PR 7/8, Her 0

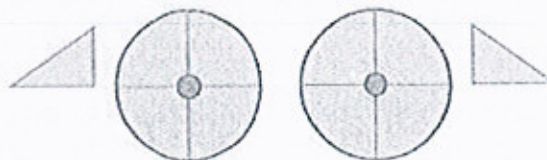
patient didnt receive AROMAZINE AFFINTOR ZOLADEX--- dp + fungating
breast mass

Recommendations:

for new CTs + bone scan
meanwhile to start gem /carbo due to fungating breast mass

MDT Outcome & Plan

In view of high burden disease with fungation of Rt breast mass and previous
endocrine resistance...Shift to Gemzar/Carboplatin Urgently.



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
<i>[Signature]</i>	<i>[Signature]</i>				

05/24/2026

Patient Name : الأستاذة / فله محمود سيد محمود عيسى :	Request No. : 13005509
Patient No. : 28403262101382	Request Date : 18/03/2026
Path No. : 751/26	Verification Date : 28/03/2026
Sex : Female	Financial Category : جوماتاف روسي - نفقة الدولة
Age : 41 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: CT-scan showing progressive course regarding right axillary lymphadenopathy in patient with history of right breast cancer.

Nature of Specimen: Right axillary lesion, Tru-cut biopsy, collected on 17/03/2026, at 13:05, cold ischemia time: 20 seconds.

Gross Examination:

- Fragmented grey white tissue cores, ranging in size between 0.2 cm & 0.6 cm long. Totally submitted.

Microscopic Examination:

* Histopathological examination reveals cores of tumor formed of malignant epithelial cells showing moderate nuclear pleomorphism and eosinophilic cytoplasm, arranged in sheets with poor tendency to tubular formation amid desmoplastic stroma. There is focal necrosis. No lymph nodal tissue identified.

Diagnosis:

Right axillary lesion, Tru-cut biopsy:

Findings are consistent with carcinomatous deposits, likely of breast origin.

- Case for further (GATA-3, ER, PR & HER2).

Signature

Dr. Walaa Ibrahim Ghonaim.

Dr. Duaa Mohamed.

Verified by :

Dr.Duaa Mohamed

Printing Date : 30/03/2026 13:04:59

Page 1 of 1

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name :	الأستاذة / فله محمود سيد محمود عيسى	Request No. :	13005509
Patient No. :	28403262101382	Request Date :	18/03/2026
Path No. :	751/26	Verification Date :	28/03/2026
Sex :	Female	Financial Category :	جوسناف روسي - نفقة الدولة
Age :	41 Y	Department :	Out Patient
Referring Doc. :		IHC No. :	
		Registered by :	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: CT-scan showing progressive course regarding right axillary lymphadenopathy in patient with history of right breast cancer.
Nature of Specimen: Right axillary lesion, Tru-cut biopsy, collected on 17/03/2026, at 13:05, cold ischemia time: 20 seconds.

Technique:
 Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER clone (1E2), PR clone (SP1), Her2 clone (4B5), GATA3 (L50-823).

Results:
 IPT (Positive control showed appropriate reactivity):
 GATA3: Diffuse strong positive staining in tumor cells.
 ER: Strong nuclear staining in 98% of tumor cells (score 8/8).
 PR: Strong nuclear staining in 50% of tumor cells (score 7/8).
 Her2: Faint incomplete membrane staining in 7% of tumor cells (Ultra-low, score 0+).

Conclusion:
 Right axillary lesion, Tru-cut biopsy:
 Findings are consistent with carcinomatous deposits of breast origin.

PT:
 GATA3: Strong positive staining in tumor cells.
 ER: Positive (score 8/8).
 PR: Positive (score 7/8).
 Her2: Ultra-low (score 0+) (Faint incomplete membrane expression in 7% of tumor cells).

Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :
 Dr.Duaa Mohamed

Printing Date : 30/03/2026 13:05:06

Page 1 of 2

Printed By : .marwa mohamed yousse



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name الأستاذ / فله محمود سيد محمود عيسى
Patient No. 28403262101382
Path No. 751/26
Sex Female
Age 41 Y
Referreing Doc.

Request No. 13005509
Request Date 18/03/2026
Verification Date 28/03/2026
Financial Category جوستاف روسي - نفقة الدولة
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim.

Dr. Duaa Mohamed.

Verified by :

Dr.Duaa Mohamed

Printing Date : 30/03/2026 13:05:06

Page 2 of 2

Printed By : .marwa mohamed yousse



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name : Fola Mahmoud Said Mahmoud

٤٨٨١
Date : 14-03-2026 08:47

Branch : Dokki

Medical No. : 332050

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعاقد مستشفيات

ISOTOPIC BONE SCAN REPORT

- **Clinical History:** A 41-year-old female patient, with history of metastatic left breast cancer and referred for follow up.
- **Prior bone scan** dated 27/10/2025 revealed metastatic osseous lesions with stationary course since 27/10/2025
- **Radioisotope:** Tc-99m MDP
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection.

Findings

Comparison is made with the prior study dated 27/10/2025:

- There is persistent radiotracer uptake within the known osseous metastatic lesions involving the sternum, dorsolumbar vertebrae (specifically DV8, DV11, LV2, LV3), right acetabulum, right femoral head/neck region, left iliac crest and left femoral shaft. These lesions demonstrate a **stationary course**, with no significant interval change in extent or intensity of uptake compared to the previous study.
- **No new foci** of abnormal radiotracer uptake are identified to suggest progressive metastatic disease.
- Age-related degenerative/arthritis changes in the spine, sternum, both shoulders, and weight-bearing joints.

Name : Fola Mahmoud Said Mahmoud

Date : 14-03-2026 08:47

Branch : Dokki

Medical No. : 332050

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعاقد مستشفيات

Impression:

- **Stable osseous metastatic disease;** persistent but unchanged tracer uptake at known metastatic sites (sternum, spine, femora).
No new lesions identified.
- Continue routine follow-up per oncology protocol.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Name : fulla mahmoud sayed mahmoud ci

Branch : Portable

Ref.Physician Prof : ct cap+c

Date : 27-01-2026 02:11

Medical No. : GRI-33668

Ins.Company :

- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Fat containing umbilical hernia is noted.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Still noted D8 ,D11 and L2 and manubrium sterni mixed lytic and sclerotic metastatic osseous lesions ,for bone scan correlation.

OPINION:

- Progressive course regarding the right axillary lymphadenopathy.
- Stationary course regarding the rest of the study.

Dr. Sherif Wagdy

Dr. Sherif Wagdy

Name : fulla mahmoud sayed mahmoud ei

Branch : Portable

Ref.Physician Prof: ct cap+c

Date : 27-01-2026 02:11

Medical No. : GRI-33668

Ins.Company :

**MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN
AND PELVIS:**

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Compared to the last study dated 28-7-2025 ,the current study revealed:

(I) Chest:

- Still seen swollen right breast with thickened trabeculations and rather stationary course regarding the previously seen LIQ and anterior chest wall complex cystic lesion , measuring 12 x7.1 cm for sonomammographic correlation.
- Progressive course since last study regarding the size of the previously seen right axillary pathological lymphadenopathies reaching 26x24 mm in today's study.
- Patent central air ways with well inflated both lungs.
- Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- Still noted mosaic attenuation of both lower lobes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

Stationary course of the previously noted:

- The liver is moderately enlarged in size showing homogenous reduced CT density with complete resolution of the segment 4a focal lesion.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.

Name : FULLA MAHMOUD

Branch : Portable

Ref.Physician Prof :

Date : 18-01-2026 11:57

Medical No. : GRI-12347

Ins.Company :

PATIENT NAME	SABAH ABDELAZIZ
REFF. BY. PROF .DR	MOHAMED HEGAZY
DATE	2 / 07 / 2024

Indication: Follow up after neoadjuvant chemotherapy of pathologically proved right breast carcinoma.

BILATERAL BREAST ULTRASONOGRAPHY REVEALED:

- Homogenous (predominantly glandular) echo-texture of both breasts.
- **The previously described pathologically proved malignant mass lesion is seen at 6 o'clock position of the left breast , zone 3 ,now measures (6.1 x 7.2 x 6.6 cm). with internal vascularity detected within**
- **another similar irregular ill defined heterogenous lesion is seen at 4 oclock zone 3 measuring 2.9 x 4.5 x 4.3 cm. With internal vascularity detected within**
- diffuse right edematous changes in form of skin thickening and echogenic fat lobules
- **No detected suspicious solid or cystic masses in the left breast.**
- Multiple pathological right level I,II,III axillary lymph nodes with lost cortical hilum , the largest measures 27 x 22 mm

CONCLUSION:

- right pathologically proven breast carcinoma with ipsilateral pathological axillary lymph nodes (BIRADS 6).
- Normal left breast (BIRADS 1 category)..

DR Farah elshobaky

Name : fulla mahmoud sayed mahmoud ei

Branch : Portable

Ref.Physician Prof : ct cap+c

Date : 27-01-2026 02:11

Medical No. : GRI-33668

Ins.Company :

- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Fat containing umbilical hernia is noted.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Still noted D8 ,D11 and L2 and manubrium sterni mixed lytic and sclerotic metastatic osseous lesions ,for bone scan correlation.

OPINION:

- Progressive course regarding the right axillary lymphadenopathy.
- Stationary course regarding the rest of the study.

Dr. Sherif Wagdy

Dr. Sherif Wagdy



مركز النيل للأشعة والتحاليل

Name : Fola Mahmoud Said Mahmoud
Branch : Dokki
Ref. Physician Prof : Goussay Roussey Egypti

Date : 25-06-2026 10:37
Medical No. : 402415

Tc-99m MDP FOLLOW UP BONE SCAN

History

41 year-old female patient with history of metastatic breast cancer to the bone received chemo-radiotherapy. complains of bony ache and referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- Considering the last bone scan report dated 14/3/2026, the current scan revealed still noted old osseous lesions which were located at the sternum, DV8, DV11, LV2, LV3, right acetabulum, left iliac crest and left femoral shaft.
- Near complete resolution of old lesion involving the right femoral head/neck region.
- No newly developed bone lesion could be detected.
- Otherwise rest of the scanned skeleton is unremarkable.

IMPRESSION

- Considering the last bone scan report dated 14/3/2026, the current scan revealed rather stationary course of the old bone lesions with resolution of old lesion involving the right femoral head/neck region as described above with no new osseous lesion could be detected.
- Follow up bone scan is advised to monitor the therapeutic response.

MUCH OBLIGED
Dr. Alaa Nagy, MD
Nuclear medicine

N I L E D I A G N O S T I C C E N T E R

Medical No. :

3921

Patient's Name:

Fola Mahmoud Said Mahmoud

Date:

2026-09-22

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



Examination:

■ Computed tomography of the chest, abdomen and pelvis

Technical data:

- 5 mm CT scans of the chest, abdomen and pelvis with IV contrast

Clinical data: known right breast cancer

Comparison: 13-06-2026

Findings:

Chest findings:

- Stationary right breast large macrolobulated parenchymal lesion 14x6.5 cm with marked skin thickening
- Stationary right axillary globular shape soft tissue lesion 2.5 cm
- Indeterminate left axillary small LNs with thickened cortices
- Clear both lungs fields with no active parenchymatous infiltrates.
- No hilar or mediastinal masses or sizable LNs.
- Normal appearance of the tracheobronchial tree.
- No evidence of pleural effusion or thickening.
- Average cardiac size, no pericardial effusion.

Abdomen and pelvis findings:

- Still noted D8, D11, L2 and sternal mixed lytic and sclerotic lesions
- Newly developed L5 lytic lesion
- The liver shows enlarged size with smooth contours, diffuse low hepatic parenchyma density (enlarged fatty liver) and no evidence of focal lesions.
- No intra hepatic biliary dilatation.
- Normal caliber and homogenous enhancement of the portal vein.
- The spleen shows normal size. No evidence of focal lesions.
- Normal CT appearance of the gall bladder, pancreas and adrenal glands
- Both kidneys are of normal site, size and shape, no stones, back pressure, cysts or masses bilaterally.
- The urinary bladder shows average distention, no wall thickening, no stones, diverticulae or masses.
- No suspicious abdominal or pelvis enlarged LNs
- No CT evidence of intestinal obstruction

Medical No. : 3921

Patient's Name: Fola Mahmoud Said Mahmoud

Date: 2026-09-22

Referred by: Prof.Dr. no doctor

Hospital: Gonstave Roussey International Egypt



- No pneumoperitoneum
- No free or localized abdominal or pelvis fluid collection

Opinion:

- Stationary right breast large macrolobulated parenchymal lesion 14x6.5 cm with marked skin thickening
- Stationary right axillary globular shape soft tissue lesion 2.5 cm
- Indeterminate left axillary small LNs with thickened cortices
- Still noted D8, D11, L2 and sternal mixed lytic and sclerotic lesions
- Newly developed L5 lytic lesion

Regards, Dr. Ahmed Rezk, MD.

Ahmed



SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Fola Mahmoud Said Mahmoud Eissa

Date : 20-09-2026 09:52

Branch : Dokki

Medical No. : 465664

Ref.Physician Prof : Goustav Roussey Egypt

تعلق مستشفيات
Ins.Company :

Tc^{99m} MDP FOLLOW UP SKELETAL SCINTIGRAPHY

History

42 year-old female patient, with history of metastatic right breast cancer to the bone, (intra-medullary nail in the left femur 2 months ago) referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

Considering the previous bone scan report dated 25/6/2026, the current study revealed regression of the metastatic bone disease evidenced by:

- Near complete disappearance of the previously depicted lesions at the sternum, DV8, DV11, LV2, LV3 and right acetabulum.
- Rather stationary course of the other lesion located at the left iliac crest.
- Enhanced radiotracer uptake along the shaft of the left femur, likely reactive for intra-medullary nail.
- No newly developed lesions could be elicited on the current study.
- Degenerative changes were also noted at different levels of the spine and around big joints.

OPINION

- Remarkable regression of the previously noted bone lesions as described above with no newly developed ones as described above indicating good response to the given therapy.

Much Obliged

Dr. Alaa Nagy, MD

Nuclear medicine

Patient's Name: Fola Mahmoud Said Mahmoud
Date: 2026-06-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA with no available DICOM images for comparison.

I- CHEST:

- Large right breast macrolobulated lesion measuring 13.9 x 6.7 cm with skin thickening associated with multiple right axillary and subpectoral lymph nodes largest measuring 26 x 22 mm for sono-mammography.
- Mild right pleural effusion with underlying right lower lobe subsegmental lung collapse.
- No pulmonary nodules, masses or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Few mixed lytic and sclerotic bone lesions are seen at D8, D11 and L2 vertebral bodies as well as the manubrium sterni.

Opinion:

Patient's Name: Fola Mahmoud Said Mahmoud
Date: 2026-06-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- ❖ Mild right pleural effusion with underlying right lower lobe subsegmental lung collapse.
- ❖ D8, D11 and L2 vertebral bodies and manubrium sterni mixed lytic and sclerotic bone lesions for bone scan.

Dr.sherif Wagdy

Dr.sherif Wagdy

Patient's Name: Fola Mahmoud Said Mahmoud
Date: 2026-06-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA with no available DICOM images for comparison.

I- CHEST:

- Large right breast macrolobulated lesion measuring 13.9 x 6.7 cm with skin thickening associated with multiple right axillary and subsectoral lymph nodes largest measuring 26 x 22 mm for sono-mammography.
- Mild right pleural effusion with underlying right lower lobe subsegmental lung collapse.
- No pulmonary nodules, masses or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Few mixed lytic and sclerotic bone lesions are seen at D8, D11 and L2 vertebral bodies as well as the manubrium sterni.

Opinion:

Patient's Name: Fola Mahmoud Said Mahmoud
Date: 2026-06-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- ❖ Mild right pleural effusion with underlying right lower lobe subsegmental lung collapse.
- ❖ D8, D11 and L2 vertebral bodies and manubrium sterni mixed lytic and sclerotic bone lesions for bone scan.

Dr.sherif Wagdy

Dr.sherif Wagdy

Patient's Name: Fola Mahmoud Said Mahmoud
Date: 2026-06-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA with no available DICOM images for comparison.

I- CHEST:

- Large right breast macrolobulated lesion measuring 13.9 x 6.7 cm with skin thickening associated with multiple right axillary and subsectoral lymph nodes largest measuring 26 x 22 mm for sono-mammography.
- Mild right pleural effusion with underlying right lower lobe subsegmental lung collapse.
- No pulmonary nodules, masses or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Few mixed lytic and sclerotic bone lesions are seen at D8, D11 and L2 vertebral bodies as well as the manubrium sterni.

Opinion:

Patient's Name: Fola Mahmoud Said Mahmoud
Date: 2026-06-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- ❖ Mild right pleural effusion with underlying right lower lobe subsegmental lung collapse.
- ❖ D8, D11 and L2 vertebral bodies and manubrium sterni mixed lytic and sclerotic bone lesions for bone scan.

Dr.sherif Wagdy

Dr.sherif Wagdy

05/24/2026

MDT FORM

Name:	Euahh Mohamad El-Sayed Mohamad	MRN:	4881
Age:	42 Y	ID:	28403262101382
Menopausal Sta		Phone:	01019079266
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: IDC
ER: 8/8 PR: 8/8 HER2: 0 KI67: 0

Sonomammography:

Mets Workup:

History:

MBC to bone and liver since 2022, Rec Taxol-Carbo 7 cycles 2022 RD
Femara + Zoladex till 2024 DP : Fasiodex+Ibrance 8-->11 2024 DP,
Xeloda 3 cycles DP .. Taxotere 12 cycles then 1-2026 DP Rt Axillary LN biopsied
acc to MDT on 2-2026: same behavior breast origin ER 8/8, PR 7/8, Her 0

patient didnt receive AROMAZINE AFFINTOR ZOLADEX--- dp + fungating
breast mass

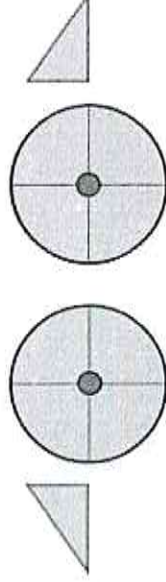
Recommendations:

for new CTs + bone scan
meanwhile to start gem /carbo due to fungating breast mass

MDT Outcome & Plan

In view of high burden disease with fungation of Rt breast mass and previous
endocrine resistance.....Shift to Gemzar/Carboplatin Urgently.

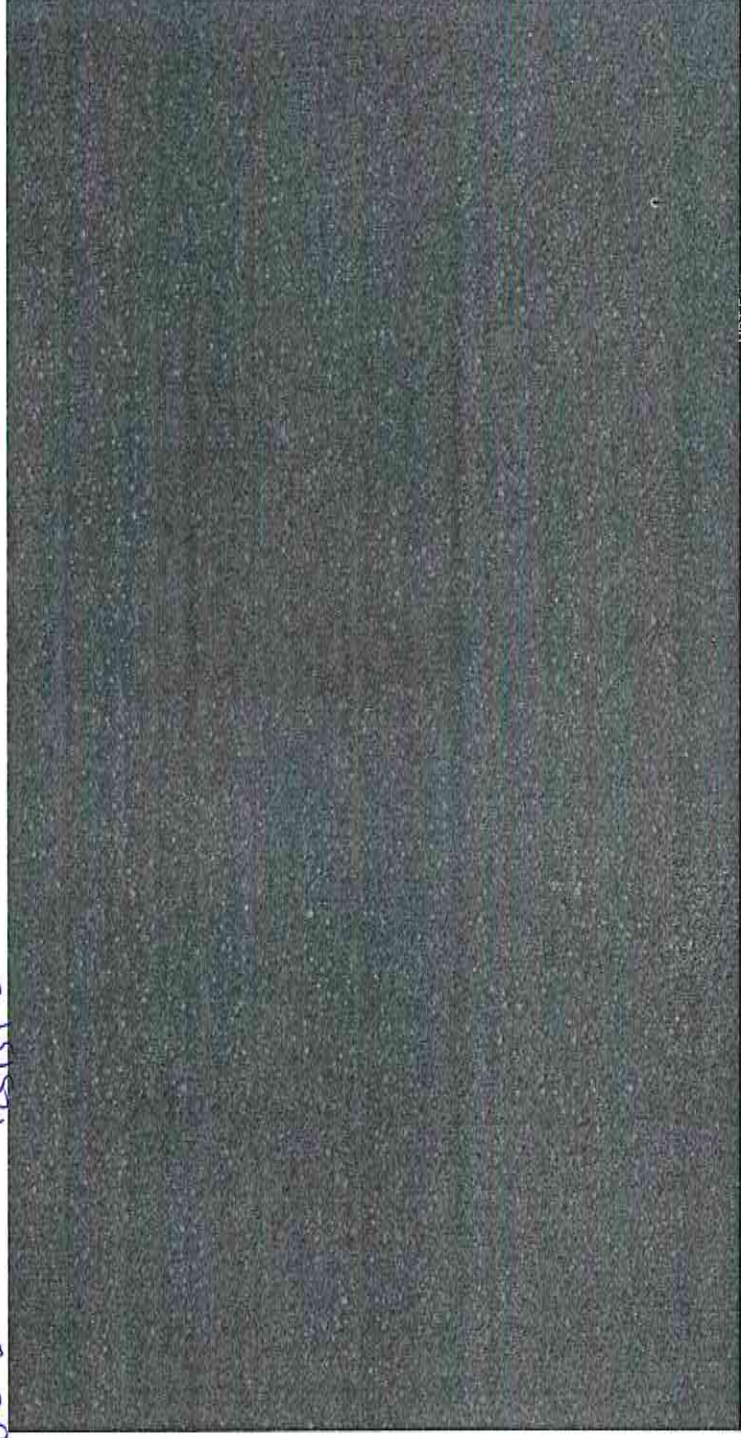
R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ شيماء لاشين	د/ شيماء عيسى	د/ شيماء عيسى			



Name : Fola Mahmoud Said Mahmoud

Date : 14-03-2026 08:47

Branch : Dokki

Medical No. : 332050

Ref.Physician Prof : Goustav Roussey Egypt

تعاقد مستشفى : Ins.Company

ISOTOPIC BONE SCAN REPORT

- **Clinical History:** A 41-year-old female patient, with history of metastatic left breast cancer and referred for follow up.
- **Prior bone scan** dated 27/10/2025 revealed metastatic osseous lesions with stationary course since 27/10/2025
- **Radioisotope:** Tc-99m MDP
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection.

Findings

Comparison is made with the prior study dated 27/10/2025:

- There is persistent radiotracer uptake within the known osseous metastatic lesions involving the sternum, dorsolumbar vertebrae (specifically DV8, DV11, LV2, LV3), right acetabulum, right femoral head/neck region, left iliac crest and left femoral shaft. These lesions demonstrate a **stationary course**, with no significant interval change in extent or intensity of uptake compared to the previous study.
- **No new foci** of abnormal radiotracer uptake are identified to suggest progressive metastatic disease.
- Age-related degenerative/arthritis changes in the spine, sternum, both shoulders, and weight-bearing joints.

Name : Fola Mahmoud Said Mahmoud
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 14-03-2026 08:47
Medical No. : 332050
Ins.Company : تعافك مستشفى

Impression:

- **Stable osseous metastatic disease;** persistent but unchanged tracer uptake at known metastatic sites (sternum, spine, femora).
- **No new lesions identified.**
- Continue routine follow-up per oncology protocol.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Name : fulla mahmoud sayed mahmoud ei	Date : 27-01-2026 02:11
Branch : Portable	Medical No. : GRI-33668
Ref.Physician Prof : ct cap+c	Ins.Company :

- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Fat containing umbilical hernia is noted.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Still noted D8 ,D11 and L2 and manubrium sterni mixed lytic and sclerotic metastatic osseous lesions ,for bone scan correlation.

OPINION:

- **Progressive course regarding the right axillary lymphadenopathy.**
- **Stationary course regarding the rest of the study.**

Dr. Sherif Wagdy

Dr. Sherif Wagdy

Name : fulla mahmoud sayed mahmoud ei

Branch : Portable

Ref.Physician Prof : ct cap+c

Date : 27-01-2026 02:11

Medical No. : GRI-33668

Ins.Company :

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Compared to the last study dated 28-7-2025 ,the current study revealed:

(I) Chest:

- Still seen swollen right breast with thickened trabeculations and rather stationary course regarding the previously seen LIQ and anterior chest wall complex cystic lesion , measuring 12 x7.1 cm for sonomammographic correlation.
- Progressive course since last study regarding the size of the previously seen right axillary pathological lymphadenopathies reaching 26x24 mm in today's study.
- Patent central air ways with well inflated both lungs.
- Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- **Still noted** mosaic attenuation of both lower lobes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

Stationary course of the previously noted:

- The liver is moderately enlarged in size showing homogenous reduced CT density with complete resolution of the segment 4a focal lesion.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.

Name : FULLA MAHMOUD

Date : 18-01-2026 11:57

Branch : Portable

Medical No. : GRI-12347

Ref.Physician Prof :

Ins.Company :

PATIENT NAME	SABAH ABDELAZIZ
REF. BY.	MOHAMED HEGAZY
PROF.DR	
DATE	2 / 07 / 2024

Indication: Follow up after neoadjuvant chemotherapy of pathologically roved right breast carcinoma.

BILATERAL BREAST ULTRASONOGRAPHY REVEALED:

- Homogenous (predominantly glandular) echo-texture of both breasts.
- *The previously described pathologically proved malignant mass lesion is seen at 6 o'clock position of the left breast , zone 3 ,now measures (6.1 x 7.2 x 6.6 cm). with internal vascularity detected within*
- *another similar irregular ill defined heterogenous lesion is seen at 4 oclock zone 3 measuring 2.9 x 4.5 x 4.3 cm. With internal vascularity detected within*
- diffuse right edematous changes in form of skin thickening and echogenic fat lobules
- *No detected suspicious solid or cystic masses in the left breast.*
- Multiple pathological right level I,II,III axillary lymph nodes with lost cortical hilum , the largest measures 27 x 22 mm

CONCLUSION:

- right pathologically proven breast carcinoma with ipsilateral pathological axillary lymph nodes (BIRADS 6).
- Normal left breast (BIRADS 1 category)..

DR Farah elshobaky

Patient Name	الأستاذ / فله محمود سيد محمود عيسى	Request No.	: 13005509
Patient No.	: 28403262101382	Request Date	: 18/03/2026
Path No.	: 751/26	Verification Date	: 28/03/2026
Sex	: Female	Financial Category	: نفقة الدولة - جوستاف روسي
Age	: 41 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: CT-scan showing progressive course regarding right axillary lymphadenopathy in patient with history of right breast cancer.
Nature of Specimen: Right axillary lesion, Tru-cut biopsy, collected on 17/03/2026, at 13:05, cold ischemia time: 20 seconds.

Gross Examination:

- Fragmented grey white tissue cores, ranging in size between 0.2 cm & 0.6 cm long. Totally submitted.

Microscopic Examination:

* Histopathological examination reveals cores of tumor formed of malignant epithelial cells showing moderate nuclear pleomorphism and eosinophilic cytoplasm, arranged in sheets with poor tendency to tubular formation amid desmoplastic stroma. There is focal necrosis. No lymph nodal tissue identified.

Diagnosis:

Right axillary lesion, Tru-cut biopsy:
Findings are consistent with carcinomatous deposits, likely of breast origin.
- Case for further (GATA-3, ER, PR & HER2).

Signature

Dr. Walaa Ibrahim Ghonaim.
Dr. Duaa Mohamed.

Verified by :
Dr.Duaa Mohamed

Printing Date : 30/03/2026 13:04:59

Page 1 of 1

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / فله محمود سيد محمود عيسى	Request No.	: 13005509
Patient No.	: 28403262101382	Request Date	: 18/03/2026
Path No.	: 751/26	Verification Date	: 28/03/2026
Sex	: Female	Financial Category	: نفقة الدولة - جوستاف روسي
Age	: 41 Y	Department	: Out Patient
Referreing Doc. :		IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: CT-scan showing progressive course regarding right axillary lymphadenopathy in patient with history of right breast cancer.
Nature of Specimen: Right axillary lesion, Tru-cut biopsy, collected on 17/03/2026, at 13:05, cold ischemia time: 20 seconds.

Technique:

Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER clone (1E2), PR clone (SP1), Her2 clone (4B5), GATA3 (L50-823).

Results:

IPT (Positive control showed appropriate reactivity):

GATA3: Diffuse strong positive staining in tumor cells.

ER: Strong nuclear staining in 98% of tumor cells (score 8/8).

PR: Strong nuclear staining in 50% of tumor cells (score 7/8).

Her2: Faint incomplete membrane staining in 7% of tumor cells (Ultra-low, score 0+).

Conclusion:

Right axillary lesion, Tru-cut biopsy:

Findings are consistent with carcinomatous deposits of breast origin.
CT:

GATA3: Strong positive staining in tumor cells.

ER: Positive (score 8/8).

PR: Positive (score 7/8).

Her2: Ultra-low (score 0+) (Faint incomplete membrane expression in 7% of tumor cells).

Scoring system: "ER, PR"

SCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :

Dr.Duaa Mohamed

Printing Date : 30/03/2026 13:05:06

Page 1 of 2

Printed By : .marwa mohamed youss



Patient Name	الأستاذة / فله محمود سيد محمود عيسى	Request No.	13005509
Patient No.	28403262101382	Request Date	18/03/2026
Path No.	751/26	Verification Date	28/03/2026
Sex	Female	Financial Category	نقطة الدولة - جوستاف روسي
Age	41 Y	Department	Out Patient
Referreing Doc.	Registered by .marwa mohamed yousse		

IMMUNOHISTOCHEMISTRY REPORT

1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim.
Dr. Duaa Mohamed.

Verified by :
Dr.Duaa Mohamed

Printing Date : 30/03/2026 13:05:06

Page 2 of 2

Printed By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

A-Patient ID:

1- Name : فاطمة محمد سيد محمد

Sex:

Female

MDT form

3- Medical number:

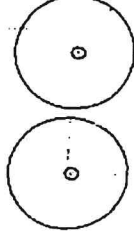
830333210

Old Med :

4065-2021

2-ID number: 28403262101382

Patient Age : 40 Y



Family history	Female History	Phone	01019079266	BMI	24.1	REASON OF MDT	REASON OF MDT
marital status	Marital Status	NRS Score	8/8	GROUP	M	Date of MDT	14/07/2024
Co morbidity	Co Morbidity	Menopausal status	Menopausal status	Initial STAGE	Initial STAGE		

Breast Imaging :				Clinical data			
Metastasis Workup :				CTCAP-->BREAST MASS+AXLN+D8 VERTEBRAL LESION			
BX	ER	PR	Her 2 Neu	SISH	KI67		
IDC	8/8	8/8	Her 2 Neu	SISH	70		
ER COMMENT	ER COMMENT	PR COMMENT	Her 2 Neu COMMENT	SISH COMMENT			
T	N	M	1 bone				
4b	1						
Sonomammography							

PATIENT HISTORY

CASE OF RT BREAST IDC CLINICALLY FUNGATING MASS OVER THE LOWER LESION OF THE RT BREAST
OFFENSIVE ODOR AND PUS
RECEIVED 7 cycles TAXOL/ carbo WITH V GOOD CLINICAL RESPONSE (neoadj.)
ct cap:RD of axillary + breast+SD of bone, multiple bone lesions
started hormonal treatment (femara+zoladex)-->PD

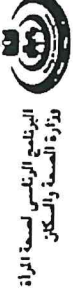
MDT OUTCOME & PLAN

- * For CDK4/6 , Faslodex & Zoladex
- * Zometa
- *Check E2 level

SIGNATURE			
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Presidential Program for Women's Health
Ministry of Health and Population



A-Patient ID:
1- Name : فلة محمود عبد محمود

Sex:

Female

2-ID number: 28403262101382

Patient Age: 39 Y

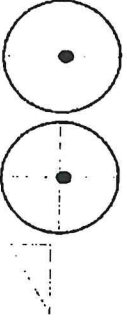
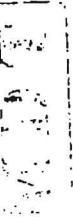
MDT form

83033210

3- Medical number:

Old Med :

4065/2021



Family history

Phone

01019079266

BMI

marital status

MRS Score

GROUP

M

Co morbidity

Menopausal status

PREMENOPAUSAL

REASON OF MDT REMDT

Date of MDT

2023-07-11

Clinical data			
Breast Imaging :		Mets Workup :	
		CTCAP-->BREAST MASS+AXLN+D8 VERTEBRAL LESION	
BX: IDC	ER: 8/8	PR: 8/8	Her 2 Neu : 0
GII RT,,,5/23			SISH : KI67 : 70
T		N	
		M	
Sonomammography			
PATIENT HISTORY			
CASE OF RT BREAST IDC CLINICALLY FUNGATING MASS OVER THE LOWER LESION OF THE RT BREAST OFFENSIVE ODOR AND PUS			
BREAST U/S--->RT BREAST SUSPICIOUS MASS			
LT BREAST DEEPLY SEATED MASS 6.5X7.5 CM			
FOR FURTHER MANGMENT			
MDT OUTCOME & PLAN			
FOR TAXOL WEEKLY			

SIGNATURE	
MEDICAL ONCOLOGY	RADIOLOGY
SURGERY	PATHOLOGY
Psychiatric	Nutrition



Presidential Program for Women's Health
Ministry of Health and Population



MDT form

A-Patient ID:

83033210

3- Medical number:

1- Name : فلة محمودة سيد محمودة

Sex:

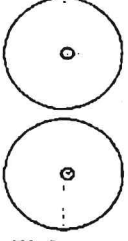
Female

Old Med :

4065/2021

2-ID number: 28403262101382

Patient Age : 39 Y



Family history Phone 01019079266 BMI 30 REASON OF MDT REMDT
marital status 1 GROUP M Date of MDT 2023-09-23 ☐
Co morbidity PREMENOPAUSAL status

Clinical data

Breast Imaging :		Mets Workup :	
		CTCAP--> BREAST MASS+AXLN+D8 VERTEBRAL LESION	
BX: IDC	ER: 8/8	PR: 8/8	Her 2 Neu : 0
GII RT...5/23			S15H : KI67 : 70
T 4B	N 1	M 1 BONE	

Sonomammography

PATIENT HISTORY

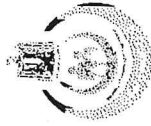
CASE OF RT BREAST IDC CLINICALLY FUNGATING MASS OVER THE LOWER LESION OF THE RT BREAST
OFFENSIVE ODOR AND PUS
RECEIVED 11 WKS TAXOL WITH V GOOD CLINICAL RESPONSE

MDT OUTCOME & PLAN

FOR TAXOL WEEKLY + CARBO D1 FOR 12 WKS THEN NEW CTS , TM
ZOPMETA /MN

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السلطة المصرية
للطب
مصر

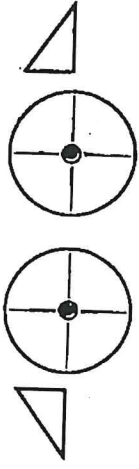
MDT FORM

7 / 7 / 20 24 G

Name	فاطمة محمد	MRN	٨٣-٣٣٣٩٠
Age		ID	٩١١٤٦٥
Menopausal status	Premenopausal	Phone	٠١-٦٣١٦٤٥١٠
NRS SCORE		Marital status	
BMI		Family history	-١٠١٩٠٧٩٥٧٧
CO morbidity		REASON OF MDT	

Clinical Data

BX:		R		L
ER:	8/8	PR:	8/8	Ki67:
		HER 2:	none	+
Sonomammography:				
Mets Workup:				



T N M

Recommendations:

Initially Locally Adv. Rt. breast IDC
S/P 7 X Tamoxifen → SP
then maintenance Femara + Celecox
recall CTCAE + follow-up → PD

MDT OUTCOME & PLAN

for new line of therapy in follow-up
8/14/24

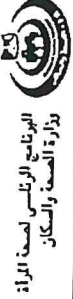
SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

SMC-DSH-F-MDT-001
Revision no:0
Issue no :1
Issue date: 1/9/2022



Presidential Program for Women's Health
Ministry of Health and Population



البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:

1- Name :

فلة محمدر سيد محمدر

Sex:

Female

83033210

Old Med :

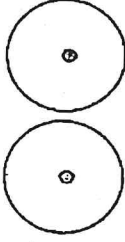
4065-2021

2-ID number:

Patient Age : 40 Y

Old Med :

4065-2021



Family history	Phone	BMI	REASON OF MDT	REASON OF MDT
	01019079266			REASON
marital status	NRS Score	GROUP	Date of MDT	Date of MDT
		M		01/12/2024
Co morbidity	Menopausal status	Initial STAGE		

Breast Imaging :			
Mets Workup :			
CTCAP-->BREAST MASS+AXLN+D8 VERTEBRAL LESION			
BX	ER	PR	Her 2 Neu
IDC	8/8	8/8	SISH
Bx COMMENT:	ER COMMENT:	PR COMMENT:	SISH COMMENT:
		-V8	

T	N	M
4b	1	1 bone
		LIVER

Sonomammography

PATIENT HISTORY

CASE OF RT BREAST IDC CLINICALLY FUNGATING MASS OVER THE LOWER LESION OF THE RT BREAST
OFFENSIVE ODOR AND PUS

RECEIVED 7 cycles TAXOL/ carbo WITH V GOOD CLINICAL RESPONSE (neoadj.)

ct cap:RD of axillary + breast+SD of bone, multiple bone lesions

started hormonal treatment (femara+zoladex)-->PD

pt rceived fasoldex+ibrance x 4 months-->disease progression

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR NEW BIOPSY from contralat. Lt axillary L.N

for Xeloda x 3 cycles then reass.

SIGNATURE			
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
	د. رانيا محمد		
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

about:blank

Patient Name: Fulla Mahmoud Sayed Mahmoud Elsa	Date : 15/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 41 Year(s)
Medical ID: 4056/2021	Pat. ID: 33668
MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:	

TECHNIQUE:

Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Compared to the last study dated 28-7-2025 ,the current study revealed:
(I) Chest:

- Patent central air ways with well inflated both lungs.
- Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- **Complete resolution** of the bilateral lower lobar calcified pulmonary nodules and subcentimetric mediastinal LNs.
- **Still noted** mosaic attenuation of both lower lobes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Swollen right breast with thickened trabeculations and LIQ and anterior chest wall complex cystic lesion , measuring 11.5x7.1 cm and right axillary pathological lymphadenopathies reaching 20x19 mm,for sonomammographic correlation.

(II) Abdomen and pelvis:

Stationary course of the previously noted:

- The liver is moderately enlarged in size showing homogenous reduced CT density with complete resolution of the segment 4a focal lesion.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.

Name : Fola Mahmoud Said Mahmoud Eissa
Branch : Dokki
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 27-10-2025 11:45
Medical No. : 235014
Ins.Company : تعاقد مستشفيات

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 41-year-old lady, with history of metastatic left breast cancer and referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

Considering the prior bone scan dated 20/03/2025:

- Still noted active skeletal lesions at sternum, DV8, DV11, LV2, LV3, Rt. femoral head/neck and left femoral shaft.
- No other abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Rather stationary course of the metastatic osseous lesions as described.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



ERM036749489

A-Patient ID:

1- Name : فقه محمود سيد محمود

Sex:

MDT form

3- Medical number:

Female

83033210

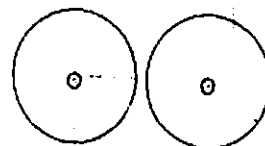
Old Med :

Old Med :

4065-2021

2-ID number: 28403262101382

Patient Age : 40 Y



Family history

Family History

Phone

Phone

01019079266

BMI

BMI

REASON OF MDT

REASON OF MDT
REMD1

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co morbidity

Co Morbidity

Menopausal
statusMenopausal Status
PREMENOPAUSAL

Initial STAGE

Initial STAGE

14/07/2024

Breast Imaging :

Clinical data

Mets Workup :

CTCAP-->BREAST MASS+AXLN+D8 VERTEBRAL LESION

BX IDC BX COMMENT	ER 8/8 ER COMMENT	PR 8/8 PR COMMENT	Her 2 Neu Her 2 Neu COMMENT: -VB	SISH SISH COMMENT	KI67 70
-------------------------	-------------------------	-------------------------	--	----------------------	------------

T 4b	N 1	M 1 bone
---------	--------	-------------

Sonomammography

PATIENT HISTORY

CASE OF RT BREAST IDC CLINICALLY FUNGATING MASS OVER THE LOWER LESION OF THE RT BREAST
OFFENSIVE ODOR AND PUS

RECEIVED 7 cycles TAXOL/ carbo WITH V GOOD CLINICAL RESPONSE (neoadj.)

ct cap:RD of axillary + breast+SD of bone, multiple bone lesions

started hormonal treatment (femara+zoladex)-->PD

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

- * For CDK4/6 , Faslodex & Zoladex
- * Zometa
- *Check E2 level

SIGNATURE			
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ كنان عزت	RADIOLOGY	RADIOLOGY د/ احمد رزق
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : فله محمود سيد محمود

Sex:

3- Medical number:

Female

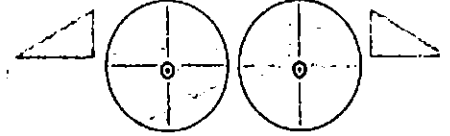
83033210

Old Med :

4065/2021

2-ID number: 28403262101382

Patient Age : 39 Y



Family history

Phone

01019079266

BMI

30

REASON OF MDT

REMDT

marital status

NRS Score

1

GROUP

M

Date of MDT

2023-09-23 ☐

Co morbidity

Menopausal
status

PREMENOPAUSAL

Clinical data

Breast Imaging :

Mets Workup :

CTCAP-->BREAST MASS+AXLN+D8 VERTEBRAL LESION

BX:
IDCER:
8/8PR:
8/8Her 2 Neu :
0

SISH :

KI67 :
70

GII RT,,,5/23

T
4BN
1M
1 BONE

Sonomammography

PATIENT HISTORY

CASE OF RT BREAST IDC CLINICALLY FUNGATING MASS OVER THE LOWER LESION OF THE RT BREAST
OFFENSIVE ODOR AND PUS
RECEIVED 11 WKS TAXOL WITH V GOOD CLINICAL RESPONSE

MDT OUTCOME & PLAN

FOR TAXOL WEEKLY+ CARBO D1 FOR 12 WKS THEN NEW CTS , TM
ZOPMETA /MN

SIGNATURE

MEDICAL ONCOLOGY

د/ محمد جمال

RADIOLOGY

د/ هبة فرج

SURGERY

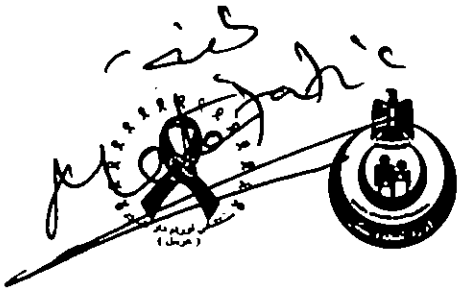
د/ احمد جمال

PATHOLOGY

Psychiatric

Nutrition

امراء حبيب



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية
صحة
مصر

٤.٦٥
٢.٩١

MDT FORM

/ / 20

G

Name	فله حور	MRN	٨٣.٣٣٩١٠
Age		ID	
Menopausal status	pregnant	Phone	٠١٩-٧٩٥٦٦
NRS SCORE		Marital status	
BMI		Family history	١١٩٥٦٣٧٨٣٨
CO morbidity		REASON OF MDT	

Clinical Data

BX:	R	L	
ER: 8/8	PR: 8/8	HER 2: -ve	Ki67: 70%
Sonomammography:			
Mets Workup:	TNM		

Recommendations:

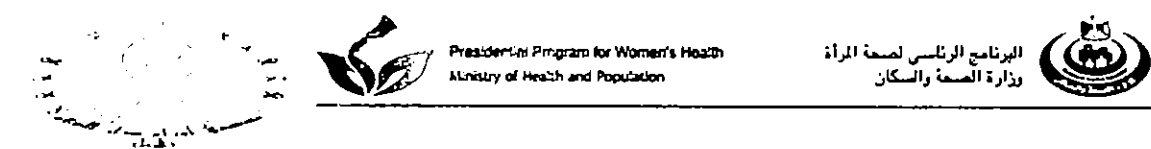
At Met Breast Ca to bone, liver
was on Ibrance / Faslodex x 4 months
recent Result CTCA P+ Murphy →
Disease progression

MDT OUTCOME & PLAN

Dr. H. H. H.
Dr. H. H. H.

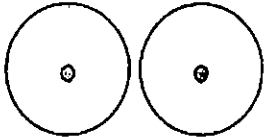
SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



MDT form

A-Patient ID: 1- Name : **فله محمود سيد محمود** Sex: Female 3- Medical number: 83033210 Old Med : 4065-2021
2-ID number: Patient Age : 40 Y



Family history	Family History	Phone	Phone	BMI	BMI	REASON OF MDT	REASON OF MDT
			01019079266				REMDT
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date of MDT
					M		01/12/2024 ☐
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE		
			PREMENOPAUSAL				

Clinical data

Breast Imaging :			Mets Workup : CTCAP-->BREAST MASS+AXLN+D8 VERTEBRAL LESION		
BX IDC BX COMMENT:	ER 8/8 ER COMMENT:	PR 8/8 PR COMMENT:	Her 2 Neu Her 2 Neu COMMENT: -V6	SISH SISH COMMENT :	KI67 70
T 4b			N 1		M 1 bone LIVER

Sonomammography

PATIENT HISTORY
CASE OF RT BREAST IDC CLINICALLY FUNGATING MASS OVER THE LOWER LESION OF THE RT BREAST
OFFENSIVE ODOR AND PUS
RECEIVED 7 cycles TAXOL/ carbo WITH V GOOD CLINICAL RESPONSE (neoadj.)
ct cap:RD of axillary + breast+SD of bone, multiple bone lesions
started hormonal treatment (femara+zoladex)-->PD
pt rceived fasoldex+ibrance x 4 months-->disease progression

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
FOR NEW BIOPSY from contralat. Lt axillary L.N
for Xeloda x 3 cycles then reass.

SIGNATURE			
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ رضوي صبره	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Presidential Program for Women's Health
Ministry of Health and Populationالبرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:

1- Name : فلة محمود سيد محمود

Sex:

3- Medical number:

Female

83033210

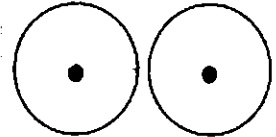
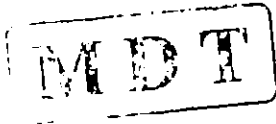
Old Med :

Old Med :

4065-2021

2-ID number: 28403262101382

Patient Age : 40 Y



Family history	Family History	Phone	Phone	BMI	BMI	REASON OF MDT	REASON OF MDT
			01019079266				REMDT
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date Of MDT
					M		14 / 07 / 2024 ☐
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE		
			PREMENOPAUSAL				

Clinical data

Breast Imaging :

Mets Workup :

CTCAP-->BREAST MASS+AXLN+D8 VERTEBRAL LESION

BX IDC	ER 8/8	PR 8/8	Her 2 Neu	SISH	Ki67 70
BX COMMENT:	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT: -ve	SISH COMMENT :	

T 4b	N 1	M 1 bone
---------	--------	-------------

Sonomammography

PATIENT HISTORY

CASE OF RT BREAST IDC CLINICALLY FUNGATING MASS OVER THE LOWER LESION OF THE RT BREAST
OFFENSIVE ODOR AND PUS

RECEIVED 7 cycles TAXOL/ carbo WITH V GOOD CLINICAL RESPONSE (neoadj.)

ct cap:RD of axillary + breast+SD of bone, multiple bone lesions

started hormonal treatment (femara+zoladex)-->PD

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

* For CDK4/6 , Faslodex & Zoladex

* Zometa

*Check E2 level

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ هنان عزت	RADIOLOGY	RADIOLOGY د/ احمد رزق
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 24/03/2025
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 41 Year(s)
Medical ID: 4065/2021	Pat. ID: 33668

segment IVb measuring about 10 mm. No intra-hepatic or extra-hepatic biliary radicles dilatation.

- Patent homogenously enhancing portal vein.
- Still seen small left paraumbilical fat containing hernia.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.

THE SCANNED SKELETON:

- Rather stationary course of the D11 lytic lesion with cortical invading and extra medullary soft tissue is noted as well as the previously noted other lytic lesions, with sternal lytic lesions show cortical erosion and extra medullary soft tissue.

OPINION:

Status follow up of right breast cancer showing:

- ***Slight progressive course*** of the right breast mass with mixed response of bilateral axillary lymph nodes, for clinical and SM correlation.
- ***Rather stationary course*** of the left hepatic lobe enhancing focal lesion.
- ***Rather stationary*** course of lytic osseous lesions as described above.
- Stationary course of the rest of the study.

Dr.Maitha Fadhl

Dr. Walid Mohamed, M.D.



Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 24/03/2025
Exam : C.T. ABDOMEN & PELIVS WITH CONTRAST	Age : 41 Year(s)
Medical ID: 4065/2021	Pat. ID: 33668

**POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

****Reported case of right breast cancer, compared to last study dated 23/10/2024 the recent study shows:**

I. CHEST

- **Slight progressive course** of the previously noted:
 - Right breast LIQ heterogeneous irregular mass lesion reaching the overlying skin currently measures about 8.8x4.6 cm compared to 8.4 x 4 cm priorly with adjacent ill-defined heterogeneous density. It is seen abutting the chest wall muscles with no definite infiltration.
 - Overlying skin thickening currently measures about 1.1cm compared to 0.8cm in prior study.
- **Mixed course** regarding the size of the bilateral pathologically enlarged axillary lymph nodes at level I&II, **some** increase in size while some of **them show decreased** in size, the largest on the right side measures currently about 2.2x1.5 cm compared to 2 x1.2 cm in the prior study and largest on the left measures about 2.3x1.7 cm compared to 1.9x1.6 cm in prior study.
- Still no sizable pulmonary masses, nodules, consolidative patches or cavitary lesions or collapse.
- No enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, thickening or masses or pericardial effusion.
- Normal CT appearance of the heart and major thoracic vessels

II. ABDOMEN AND PELVIS FINDINGS:

- The liver is still seen moderately enlarged with diffuse homogeneous low CT parenchyma and regular outline. **Rather stationary course** of the previously noted left hepatic lobe small rounded enhancing focal lesion seen at



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة

Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 18/06/2022
Exam : Item 1	Age : 39 Year(s)
Medical ID: 2021/4065	33668

- The scanned skeleton shows D8 vertebral body lytic lesion, measuring about 13x9 mm.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

A known case of locally advanced right breast cancer showing:

- ***Large medial right breast mass with skin attachment and abutting the chest wall muscles.***
- ***Pathological right axillary and internal mammary lymph nodes.***
- ***D8 vertebral body lytic lesion for bone scan correlation.***
- ***Fatty hepatomegaly showing heterogeneous parenchyma for further assessment.***
- ***No pulmonary metastatic deposits.***

Dr. Wafaa Ali

Dr. Ofa Magdy, M.D.

Specialist: Dr. Hossam El-Salam	Date : 18/06/2023
Patient Name: Fulla Nahmoud Sayed Mahmoud Eisa	Age : 39 Year(s)
Exam: Item 1	33668
Medical ID: 2021/4065	

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Reported case of right breast cancer shows:

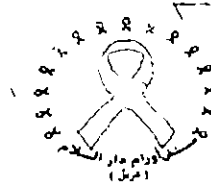
1. Chest:

- The right breast shows large mass lesion reaching skin measuring about 88x54 mm, with diffuse skin thickening. It is seen abutting the chest wall muscles.
- Pathologically enlarged right axillary lymph nodes at level I&II, the largest measures about 33x20 mm at level I.
- Prominent left axillary lymph nodes at level I the largest measures about 14 x 19 mm.
- Prominent right internal mammary lymph node measures about 11 x 7 mm.
- No sizable masses, nodules, consolidation patches or cavitary lesions.
- Prominent mediastinal lymph nodes.
- No evidence of pleural effusion or masses seen.
- Average cardiac size. No pericardial effusion detected.

2. Abdomen and pelvis:

- The liver is enlarged in size with low heterogeneous CT texture.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:



Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 18/06/2023
Exam : Item 1	Age : 39 Year(s)
Medical ID: 2021/4065	33668

- The scanned skeleton shows D8 vertebral body lytic lesion, measuring about 13x9 mm.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

A known case of locally advanced right breast cancer showing:

- ***Large medial right breast mass with skin attachment and abutting the chest wall muscles.***
- ***Pathological right axillary and internal mammary lymph nodes.***
- ***D8 vertebral body lytic lesion for bone scan correlation.***
- ***Fatty hepatomegaly showing heterogeneous parenchyma for further assessment.***
- ***No pulmonary metastatic deposits.***

Dr. Wafaa Ali

Dr. Ola Magdy, M.D.



Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 18/06/2023
Exam : Item 1	Age : 39 Year(s)
Medical ID: 2021/4065	33668

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Reported case of right breast cancer shows:

1. Chest:

- The right breast shows large mass lesion reaching skin measuring about 88x54 mm, with diffuse skin thickening. It is seen abutting the chest wall muscles.
- Pathologically enlarged right axillary lymph nodes at level I&II, the largest measures about 33x20 mm at level I.
- Prominent left axillary lymph nodes at level I the largest measures about 14 x 19 mm.
- Prominent right internal mammary lymph node measures about 11 x 7 mm.
- No sizable masses, nodules, consolidation patches or cavitary lesions.
- Prominent mediastinal lymph nodes.
- No evidence of pleural effusion or masses seen.
- Average cardiac size. No pericardial effusion detected.

2. Abdomen and pelvis:

- The liver is enlarged in size with low heterogeneous CT texture.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:



Patient Name:	Follah Mahmoud Sayed Mahmoud	Patient Gender:	Female
Patient Code:	Q37145	DATE:	7/6/2023 9:14:21 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- ✗ 38-year-old female patient with history of right breast cancer.

Technique

- ✗ Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- ✗ Increased osseous tracer uptake is seen involving **DV9/DV8** and head/neck of right femur and possibly left sacro-iliac region.
- ✗ NB: Warm soft tissue uptake is seen related to the right breast (site of known primary).

Opinion

- ✗ Few equivocal osseous lesions as described above, metastatic nature is a high possibility, for further radiological correlation and follow-up.

Dr. Omnia Alaa

Dr. Mariam Mahmoud, MD



Total DLP (mGy.cm):

Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 11/06/2024
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID: 4065/2021	Pat. ID: 33668

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.
- 3. **The scanned skeleton:**
 - The scanned skeleton shows **more sclerosis** of the right femoral lytic lesion, anterior mid sternal rarefaction,
 - **More sclerosis** of D8 and D7 lesions.
 - **Increase in lytic tumoral burden** of D11 lesion.

OPINION:

Status follow up of right breast cancer showing:

- ***Progressive course of the right breast mass.***
- ***Mixed response of the osseous lesions as described***
- ***Stationary course of the rest of the study.***

Signature Dr. Mohamed Mohsen

20/06/2024 06:34 م



Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 18/06/2023
Exam : Item 1	Age : 39 Year(s)
Medical ID: 2021/4065	33668

- The scanned skeleton shows D8 vertebral body lytic lesion, measuring about 13x9 mm.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

A known case of locally advanced right breast cancer showing:

- ***Large medial right breast mass with skin attachment and abutting the chest wall muscles.***
- ***Pathological right axillary and internal mammary lymph nodes.***
- ***D8 vertebral body lytic lesion for bone scan correlation.***
- ***Fatty hepatomegaly showing heterogeneous parenchyma for further assessment.***
- ***No pulmonary metastatic deposits.***

Dr. Wafaa Ali

Dr. Ola Magdy, M.D.



Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 18/06/2023
Exam : Item 1	Age : 39 Year(s)
Medical ID: 2021/4065	33668

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Reported case of right breast cancer shows:

1. Chest:

- The right breast shows large mass lesion reaching skin measuring about 88x54 mm, with diffuse skin thickening. It is seen abutting the chest wall muscles.
- Pathologically enlarged right axillary lymph nodes at level I&II, the largest measures about 33x20 mm at level I.
- Prominent left axillary lymph nodes at level I the largest measures about 14 x 19 mm.
- Prominent right internal mammary lymph node measures about 11 x 7 mm.
- No sizable masses, nodules, consolidation patches or cavitary lesions.
- Prominent mediastinal lymph nodes.
- No evidence of pleural effusion or masses seen.
- Average cardiac size. No pericardial effusion detected.

2. Abdomen and pelvis:

- The liver is enlarged in size with low heterogeneous CT texture.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:



Total DLP (mGy.cm):

Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 27/11/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 39 Year(s)
Medical ID: 4065/2021	33668

POS-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Reported case of right breast cancer, compared to last study dated 7/09/2023 the recent study shows:

1. Chest:

- Regressive course of the right breast shows large mass lesion reaching skin measuring about 2.5x7.6 cm compared to 9x5 cm, with diffuse skin thickening which also regressed from 1.7 to 1.1 cm. It is seen abutting the chest wall muscles.
- Mild regression of size of ipsilateral pathologically enlarged axillary lymph nodes at level I&II, the largest measures about 1.5 x1 cm compared to 3x1.5 cm at level I.
- Still noted prominent left axillary lymph nodes at level I the largest measures about 1.4x0.7 cm compared to 2.3x1.2 cm.
- Resolution of right internal mammary lymph node.
- Still no sizable masses, nodules, consolidation patches or cavitary lesions.
- No current enlarged mediastinal lymph nodes.
- No evidence of pleural effusion or masses seen.
- Average cardiac size. No pericardial effusion detected.

2. Abdomen and pelvis:

- The liver is enlarged in size with low heterogeneous CT texture. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein.
- Still seen small left paraumbilical hernia containing omentum.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.



Total DLP (mGy.cm):

Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 27/11/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 39 Year(s)
Medical ID: 4065/2021	33668

- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- The scanned skeleton shows still noted right femoral lytic lesion, anterior mid sternal rarefaction, D7 & D11 vertebral body lytic lesions.

OPINION:

Status follow up of right breast cancer showing:

- ***Regression of right breast mass, skin thickening and size of bilateral axillary lymph nodes.***
- ***Stationary course of the rest of the study.***

Dr. Shaimaa Megahed, MSc

Dr. Amr Farouk, M.D.



Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 23/10/2024
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID: 4056/2021	Pat. ID: 33668

POS-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Reported case of right breast cancer, compared to last study dated 11/06/2024 the recent study shows:

CHEST FINDINGS:

- **Progressive course** of the previously noted right breast LIQ heterogeneous irregular mass lesion reaching the overlying skin measuring about 3.2 x 8.5 cm compared to 2.8 x 7.2 cm in the previous study with adjacent ill-defined heterogeneous density. It is seen abutting the chest wall muscles with no definite infiltration.
- Rather stationary course of the overlying skin thickening
- Progressive course of the size of bilateral pathologically enlarged axillary lymph nodes at level I&II, the largest on the right side measures about 2 cm compared to 1.6 cm in the previous study and largest on the left measuring about 1.9 cm compared to 1.7 cm in the previous study.
- Still no sizable pulmonary masses, nodules, consolidative patches or cavitary lesions or collapse.
- No enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, thickening or masses or pericardial effusion.
- Normal CT appearance of the heart and major thoracic vessels

ABDOMEN AND PELVIS FINDINGS:

- The liver is enlarged in size with diffuse homogeneous low CT parenchyma and regular outline. Progressive course of the previously noted left hepatic lobe small rounded enhancing focal lesion seen at segment IVb measuring about 10 mm compared to 6 mm at last study. No intra-hepatic or extra-hepatic biliary radicles dilatation.



Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 23/10/2024
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID: 4056/2021	Pat. ID: 33668

- Patent homogenously enhancing portal vein.
- Still seen small left paraumbilical fat containing hernia.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.

THE SCANNED SKELETON:

- Mild progressive course of the D11 lytic lesion with depressed cortical interruption of the upper end plate
- Stationary course of the rest of lytic lesions

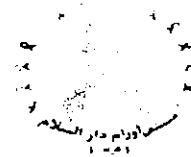
OPINION:

Status follow up of right breast cancer showing:

- Progressive course of the right breast mass as well as bilateral axillary lymph nodes.
- Progressive course of the left hepatic lobe enhancing focal lesion.
- Progressive course of D11 lytic osseous lesion.
- Stationary course of the rest of the study.

Dr.mona ali

Dr. Ibrahim Abo El Magd, M.D.



Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 17/11/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 40 Year(s)
Medical ID: 4065/2024	Pat. ID: 33668

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Right breast mass

Technique

- Bilateral MLC and CC views
- Bilateral 3D MLO DRT (Digital Breast Tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right edema pattern (increased skin thickening and coarse trabeculation)
- Right LIQ speculated dense mass lesion seen involving overlying skin and nipple
- Left benign macrocalcifications
- No left suspicious microcalcifications or architectural distortion detected.

Complementary ultrasound:

- Right increased skin thickening reaching about 1cm with dilated subdermal lymphatics
- Intra-areolar lymphatic edema
- Large ill defined hyperechoic soft tissue mass seen at 5 to 6 O'clock mass , measured roughly about 5x2.8 cm with overlying skin invasion
- Normal left skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Multiple right axillary enlarged LNs level I with irregular cortical thickening and effaced fatty hilum
- Pathologically enlarged two left axillary LNs with effaced fatty hila.
- No left breast mass

Opinion:

- Right LIQ Large centrally located mass associated with ipsilateral right axillary level I LNs BIRADS 6
- Pathological left axillary LNs likely metastatic

Signature Dr. basma mohmaed



Total DLP (mGy.cm):

Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 11/06/2024
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID: 4065/2021	Pat. ID: 33668

**POS-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

**Reported case of right breast cancer, compared to last study
dated 27/11/2023 the recent study shows:**

1. Chest:

- **Progressive course** with more appreciated central enhancement the right breast LIQ large mass lesion reaching skin measuring about 2.5x7.6x3.6 cm. It is seen abutting the chest wall muscles.
- Rather stable of size of ipsilateral pathologically enlarged axillary lymph nodes at level I&II, the largest measures about 1.5 x1 cm compared to 3x1.5 cm at level I.
- Still noted prominent left axillary lymph nodes at level I the largest measures about 1.4x0.7 cm compared to 2.3x1.2 cm.
- Still no sizable pulmonary masses, nodules, consolidative patches or cavitary lesions.
- No current enlarged mediastinal lymph nodes.
- No evidence of pleural effusion or masses seen.
- Average cardiac size. No pericardial effusion detected.

2. Abdomen and pelvis:

- The liver is enlarged in size with low heterogeneous CT texture. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein.
- Still seen small left paraumbilical hernia containing omentum.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.

MAHMOUD, FOLLAH Patient ID: 10110224003507 DOB: 26-Mar-1984 Date: 01-Jul-2024

Patient Lastname: Mahmoud
Patient Firstname: Follah
HIS Patient ID: 10110224003507
RIS Patient ID: 355552
Date of Birth: 26/03/1984

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (01/07/2024)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

Compared to previous bone scan REPORT dated 12/10/2024 done outside the NCI:

" Still seen faint heterogeneous tracer uptake over DV8/9 and right femoral head/neck.

" Regression of the previously reported left sacro-iliac region active tracer uptake.

" Newly seen active focal tracer uptake over DV11 of suspicious nature?? Metastatic for CT cuts correlation.

" Heterogeneous tracer uptake is seen over the rest of the dorso-lumbar vertebrae, likely pronounced degenerative changes for CT cuts correlation.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized.

Conclusion:

Follow up bone scan for a known case of right breast cancer shows mixed response to therapy as follows:

" Newly developed suspicious DV11 osseous lesion for CT cuts correlation.

" Still seen warm heterogeneous tracer uptake over DV8/9 and right femoral head/neck.

" Regression of the old left sacro-iliac region increased tracer uptake.

Much Obligated
Mayadah Ashraf
Mohamed Abdeltawab, MSc

===== REPORT TEXT ENDE =====

Patient Name	: الأستاذة / فله محمود سيد محمود عيسى	Request No.	: 13002347
Patient No.	: 28403262101382	Request Date	: 03/12/2024
Path No.	: 4741/24	Verification Date	: 09/12/2024
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 40 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data:

Left enlarged axillary lymph node.

Nature of specimen:

Tru cut biopsy, collected at 3/12/2024.

Gross examination:

Fragmented greyish white tissue cores measured collectively
0.5x0.5 cm, totally submitted.

Microscopic examination:

Histopathological examination of sections prepared from the lymph node biopsy received reveals complete effacement of the normal nodal architecture. There is replacement by a malignant growth made up of solid sheets and cords of small-sized malignant cells with round nuclear contour and a small amount of cytoplasm. Cellular and nuclear atypia are minimal. Mitotic figures are occasional.

Diagnosis:

Left enlarged axillary lymph node, tru cut biopsy:
Compatible with metastatic carcinoma, Grade II.
For further IHC (GATA3, ER, PR, HER2, Ki67).

Prof. Gamal Abdelatty
Dr. Noura Hefny

Verified by :

dr.esraa magdy

Printing Date : 29/12/2024 09:48:03

Page 1 of 1

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	: الأستاذة / فله محمود سيد محمود عيسى	Request No.	: 13004904
Patient No.	: 28403262101382	Request Date	: 16/05/2023
Path No.	: 1481/23	Verification Date	: 28/05/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 39 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast lesion

Nature of the specimen:

- Tru cut biopsy collected 16-5-2023.

Gross Examination:

- Three greyish white tissue cores up to 1 cm . Totally submitted .
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G III(3+3+2) (REVISED)
- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
- PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 70% nuclear reaction of tumor cells.

Diagnosis:

Right breast lesion , Tru cut biopsy:

- Invasive duct carcinoma grade II

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 70% of tumor cells.

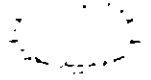
Verified by :

walaa ghoniem

Printing Date : 04/07/2023 13:42:06

Page 1 of 2

Printed By : marwa mohamed yousse



Patient Name	الأستاذة / فله محمود سيد محمود عيسى	Request No.	13002347
Patient No.	28403262101382	Request Date	03/12/2024
Path No.	4741/24	Verification Date	28/12/2024
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	40 Y	Department	Out Patient
Referreing Doc.			
		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+ based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Prof.Gamal Abdelatty.
Dr. Rofaya Abdallah.

Verified by :
dr.esraa magdy

Printing Date : 29/12/2024 09:48:08

Page 2 of 2

Printed By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadenia - Cairo, Egypt



Patient Name	: الأستاذة / فله محمود سيد محمود عيسى	Request No.	: 13002347
Patient No.	: 28403262101382	Request Date	: 03/12/2024
Path No.	: 4741/24	Verification Date	: 28/12/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Left enlarged axillary lymph node.

Nature of Specimen:

Tru-cut biopsy collected at 3/12/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 30% nuclear reaction of tumor cells.

Conclusion:

Left enlarged axillary lymph node, Tru-cut biopsy:
Metastatic carcinoma of primary mammary origin, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 30% of tumor cells.

Verified by :

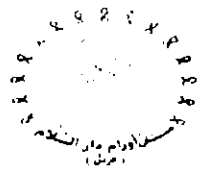
dr.esraa magdy

Printing Date : 29/12/2024 09:48:08

Page 1 of 2

Printed By : Mrs.Mayada Hegazy





Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 07/09/2023
Exam : C.T ABDOMEN & PELVIS	Age : 39 Year(s)
Medical ID: 4065/2021	Pat. ID: 33668

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

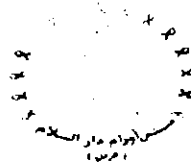
Reported case of right breast cancer, compared to last study dated 18/06/2023 the recent study shows:

1. Chest:

- Rather stationary course of the right breast shows large mass lesion reaching skin measuring about 90x50 mm, with diffuse skin thickening. It is seen abutting the chest wall muscles.
- Mild regression of size of ipsilateral pathologically enlarged axillary lymph nodes at level I&II, the largest measures about 30x15 compared to last study 33x20 mm at level I.
- Still noted prominent left axillary lymph nodes at level I the largest measures about 23x12 mm compared to last study 14 x 19 mm.
- Still noted prominent right internal mammary lymph node measures about 11 x 7 mm.
- No sizable masses, nodules, consolidation patches or cavitary lesions.
- Prominent mediastinal lymph nodes.
- No evidence of pleural effusion or masses seen.
- Average cardiac size. No pericardial effusion detected.

2. Abdomen and pelvis:

- The liver is enlarged in size with low heterogeneous CT texture. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.



Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 07/09/2023
Exam : C.T ABDOMEN & PELVIS	Age : 39 Year(s)
Medical ID: 4065/2021	Pat. ID: 33668

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections. ✖

3. The scanned skeleton:

- The scanned skeleton shows still noted right femoral lytic lesion, anterior mid sternal lytic lesion /rarefaction, enlarging D7 vertebral body lytic lesion, with newly developed D11 lytic lesion 
- No vertebral structural collapse or extra-osseous soft tissue component.

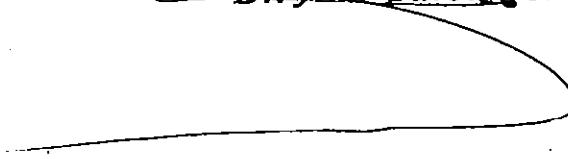
OPINION:

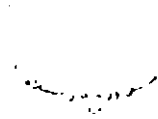
A known case of locally advanced right breast cancer showing:

- ***Newly developed D11 vertebral body lytic.***
- ***Mild regression of ipsilateral pathologically enlarged axillary lymph nodes.***
- ***Stationary course of the rest of the study.***

Dr. Rasha ElQaffas, ArBR, FEBR

Dr. Amr Farouk, M.D.





Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 12/10/2023
Exam : ultrasound ABDOMEN& PELVIS	Age : 39 Year(s)
Medical ID: 4065/2021	Pat. ID: 33668

ABDOMINO-PELVIC SCAN OF YOUR PATIENT REVEALED:

LIVER

- Enlarged size measuring 21.8 cm showing homogeneous parenchymal echo-pattern, smooth outlines & normal hepatic veins.
- Average portal vein caliber with patent lumen.
- No focal lesions. No IHBRD. No ascites.

GALL BLADDER

- Contracted at time of examination
- Average caliber of C.B.D. with no detectable stones.

SPLEEN: average size with normal echogenicity & no focal lesions.

KIDNEYS

- Normal size, parenchymal thickness and echogenicity.
- No stones or backpressure changes could be noted.

URINARY BLADDER: average wall thickness. No stones or masses.

Uterus: AVF. average size with homogenous myometrium. Endometrium thickness 0.6 cm. No focal lesions

No evidence of adnexal cystic or solid lesions

OPINION:

- ***Bright fatty liver for lab correlation***

Dr/Lamiaa Elsharkawy, MD.

Signature Dr. Lamyaa Nagah

12/10/2023 12:30 PM

Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 12/10/2023
Exam : ultrasound ABDOMEN& PELVIS	Age : 39 Year(s)
Medical ID: 4065/2021	Pat. ID: 33668

ABDOMINO-PELVIC SCAN OF YOUR PATIENT REVEALED:

LIVER

- Enlarged size measuring 21.8 cm showing homogeneous parenchymal echo-pattern, smooth outlines & normal hepatic veins.
- Average portal vein caliber with patent lumen.
- No focal lesions. No IHBRD. No ascites.

GALL BLADDER

- Contracted at time of examination
- Average caliber of C.B.D. with no detectable stones.

SPLEEN: average size with normal echogenicity & no focal lesions.

KIDNEYS

- Normal size, parenchymal thickness and echogenicity.
- No stones or backpressure changes could be noted.

URINARY BLADDER: average wall thickness. No stones or masses.

Uterus: AVF, average size with homogenous myometrium. Endometrium thickness 0.6 cm. No focal lesions

No evidence of adnexal cystic or solid lesions

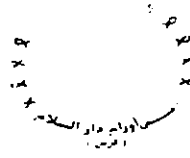
OPINION:

• *Bright fatty liver for lab correlation*

Dr/Lamiaa Elsharkawy, MD.

Signature Dr. Lamyaa Nagah

12/10/2023 12:30 PM



Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 14/05/2023
Exam : ultrasound BREAST (BOTH SIDE)	Age : 39 Year(s)
Medical ID: 4065/2021	Pat. ID: 33668

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows LIQ spiculated irregular mass lesion, of high density deeply seated at zone C.
- Complementary ultrasound revealed left breast irregular spiculated hypoechoic mass lesion deeply seated inseparable from the chest wall measures 6.5 x 7.5 cm with adjacent cutaneous nodule.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Pathological right axillary lymph nodes at level I and II largest measures 2 x 3.5 cm.
- Nonspecific left axillary LN.

Opinion:

- Left breast LIQ locally advanced malignant looking mass lesion (BIRADS 5) for histopathological correlation.
- Pathological right axillary LNs.
- Benign findings left breast (BIRADS 2)

Yours truly

Signature Dr. Aya Deabes

14/05/2023 1:40 PM



Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 07/09/2023
Exam : C.T ABDOMEN & PELVIS	Age : 39 Year(s)
Medical ID: 4065/2021	Pat. ID: 33668

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

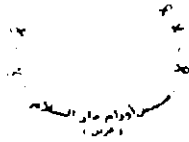
Reported case of right breast cancer, compared to last study dated 18/06/2023 the recent study shows:

1. Chest:

- Rather stationary course of the right breast shows large mass lesion reaching skin measuring about 90x50 mm, with diffuse skin thickening. It is seen abutting the chest wall muscles.
- Mild regression of size of ipsilateral pathologically enlarged axillary lymph nodes at level I&II, the largest measures about 30x15 compared to last study 33x20 mm at level I.
- Still noted prominent left axillary lymph nodes at level I the largest measures about 23x12 mm compared to last study 14 x 19 mm.
- Still noted prominent right internal mammary lymph node measures about 11 x 7 mm.
- No sizable masses, nodules, consolidation patches or cavitary lesions.
- Prominent mediastinal lymph nodes.
- No evidence of pleural effusion or masses seen.
- Average cardiac size. No pericardial effusion detected.

2. Abdomen and pelvis:

- The liver is enlarged in size with low heterogeneous CT texture. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.



Patient Name: Fulla Mahmoud Sayed Mahmoud Eisa	Date : 07/09/2023
Exam : C.T ABDOMEN & PELVIS	Age : 39 Year(s)
Medical ID: 4065/2021	Pat. ID: 33668

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.
- 3. **The scanned skeleton:**
- The scanned skeleton shows still noted right femoral lytic lesion, anterior mid sternal lytic lesion /rarefaction, enlarging D7 vertebral body lytic lesion, with newly developed D11 lytic lesion
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

A known case of locally advanced right breast cancer showing:

- ***Newly developed D11 vertebral body lytic.***
- ***Mild regression of ipsilateral pathologically enlarged axillary lymph nodes.***
- ***Stationary course of the rest of the study.***

Dr. Rasha ElQaffas, ArBR, FEBR

Dr. Amr Farouk, M.D.

مركز فؤاد دار السلام
عزیز سید

Pat. Name: فہم محمود سید

Date: 29/09/2021

REPORT

REAL TIME HIGH RESOLUTION ULTRASONOGRAPHY OF BOTH BREASTS

REVEALED:

- **Both breasts** show echogenic parenchyma with dilated central ducts, some shows moving echos, others are seen clear, changes consistent with lactation
- Right breast is the seat of mild increased skin thickening with 4 o'clock irregular heterogenous collection measuring about 43 x 20.5 with internal vascularity seen within.
- No solid or cystic mass lesions.
- Normal appearance of the skin and subcutaneous tissues.
- Normal nipple-areola complex.
- Left Nonspecific axillary lymph nodes.
- Right prominent axillary lymph nodes, one of them with diffuse cortical thickening reaching to 5mm with preserved fatty hilum.

OPINION:

- **Right 4 o'clock likely inflammatory process for proper treatment and follow up, if not response to treatment do biopsy to exclude any suspicious lesions. (BIRADS 3)**
- **Bilateral normal lactational changes (BIRADS 2)**

Best regards
Dr. Nehal Hamdey, FEBR, MSC.

MAHMOUD, FOLLAH Patient ID: 10110224003507 DOB: 26-Mar-1984 Date: 01-Jul-2024

Patient Lastname: Mahmoud

Patient Firstname: Follah

HIS Patient ID: 10110224003507

RIS Patient ID: 355552

Date of Birth: 26/03/1984

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (01/07/2024)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of

99mTc MDP Revealed:

Compared to previous bone scan REPORT dated 12/10/2024 done outside the NCI:

" Still seen faint heterogeneous tracer uptake over DV8/9 and right femoral head/neck.

" Regression of the previously reported left sacro-iliac region active tracer uptake.

" Newly seen active focal tracer uptake over DV11 of suspicious nature?? Metastatic for CT cuts correlation.

" Heterogeneous tracer uptake is seen over the rest of the dorso-lumbar vertebrae, likely pronounced degenerative changes for CT cuts correlation.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized.

Conclusion:

Follow up bone scan for a known case of right breast cancer shows mixed response to therapy as follows:

" Newly developed suspicious DV11 osseous lesion for CT cuts correlation.

" Still seen warm heterogeneous tracer uptake over DV8/9 and right femoral head/neck.

" Regression of the old left sacro-iliac region increased tracer uptake.

Much Obligated

Mayadah Ashraf

Mohamed Abdeltawab, MSc

===== REPORT TEXT ENDE =====